

Response and Proposed Allocation of Questions

Request for Medical Information dated 31 July 2026

Employee: Cory Lea Shepherd, AO3, Switchboard Services, Logan Hospital

Reference: MSH-INJ-5795 · CLM-317073

Request signed by: Mr Scott Hughes, Director, Corporate Services, LBHS — 31 July 2026

Response date: _____

IN SUMMARY

1. I am engaging with this process and will obtain and provide the information sought.
2. Four of the nine questions are not medical questions at all. Questions 2, 3, 6 and 9 ask my doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that it answer those itself, after which the properly medical part of each can be put to my practitioners.
3. Of the remainder, Questions 1(c), 1(d), 4, 5 and 8 are for my general practitioner, and Questions 1(a), 1(d), 7 and 8 require or are better answered by my treating psychiatrist. I will obtain all of them. Dr Ma's understanding of my psychiatric condition derives from my psychiatrist's correspondence to him, which is why several questions are better put to the specialist directly — and is the answer to Question 1(c).
4. Question 1(a) I answer openly in this document. Question 1(b) alone I ask be withdrawn: it asks a general practitioner for a psychiatric causal formulation, and it goes to a matter in issue in WC/2024/227.
5. I ask to be returned to my roster now. The restriction in issue asks that complaints be redirected rather than absorbed — which is the process the Health Service says it already operates, and which the Role Description does not require of me in any event.
6. I ask for an extension of the seven days, and explain at Part 8 why that period cannot be met on the Health Service's own question set.
7. I ask who the delegate is, and that any decision about my return be made by an officer outside Corporate Services.
8. My proposal for returning to work is at Part 7. Part 10 lists what I ask the Health Service to confirm.

PART 1 — THE ORIGIN OF THE REQUEST, AND THE TEST APPLIED

1.1 THE ORIGIN AND STATED PURPOSE OF THE EMPLOYEE CAPABILITIES CHECKLIST

The letter of 31 July 2026 records the origin of the checklist in the Health Service's own words:

"your recently completed Employee Capabilities Checklist (ECC) (Attachment 1) completed by Dr Day Hong Ma on the 3 July 2026 which was provided to us to facilitate a Return to Work Plan."

That is correct, and the sequence was as follows.

- (a) On 2 July 2026 I was told by the Manager, Switchboard Services that my return to work could not be facilitated until a completed Employee Capabilities Checklist was provided, and that it was required before 2:00pm on Friday 3 July 2026.
- (b) I obtained the checklist and provided it on 3 July 2026. It certified me fit to work with restrictions.
- (c) Within the hour I was told I could not attend the workplace, and my rostered shifts were charged to my accrued leave. I have not worked since.

The checklist was therefore required by the Health Service, for a purpose the Health Service has itself stated — to facilitate a return to work — and, once provided, became the basis on which work was refused. It did not certify me unfit. It certified me fit with restrictions.

I record this because it bears directly on the present request. The Health Service now seeks further information about a document it required me to obtain in order to work, and which certified that I could.

I am willing to provide clarification of that document. I ask that the clarification be directed to identifying the adjustments which allow me to work — the purpose for which the document was obtained, the purpose stated in the letter of 31 July 2026, and the purpose contemplated by Human Resources Policy G03.

1.2 THE TEST APPLIED

The letter to Dr Ma states the purpose of the request:

"to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position."

Human Resources Policy G03 (QH-POL-210), which the Health Service has cited, sets the limit at clause 3:

"Reasonable adjustment may need to be supported by medical or rehabilitation information, where necessary, to either identify required workplace adjustments and/or assess the suitability of proposed adjustments."

I have applied that test to each question: is this information necessary to identify a required adjustment, or to assess the suitability of a proposed one — and is the person being asked the right person to answer it?

Applying that test, the nine questions divide four ways. I am not declining to provide information. I am proposing that each question be directed to whoever is able to answer it.

1.3 THE CHECKLIST ALREADY ANSWERS MOST OF WHAT IS NOW ASKED

The Employee Capabilities Checklist is Attachment 1 to the Health Service's own letter. Read in full, it already answers, in substance, most of the nine questions.

Q1(d) foreseeable risk and necessary controls — answered at Additional Comments 3 and 4, which set out the recommended work pattern with clinical reasoning for each shift type, and five specific workplace modifications.

Q4 restrictions, clinical basis in functional terms, duration and review — answered at Additional Comments 2: "Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3-6 months, subject to treatment response and 8-weekly review." The review date is given on the first page as 28 August 2026.

Q5 what "complaint handling" encompasses — answered: "complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd."

Q6 the requirements of the role — answered: "Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below."

Q7 working memory in functional terms — addressed under Memorising information ("Working memory affected under stress") and at Additional Comments 1 ("reduced concentration and working memory under sustained high-demand load"), with the roster controls which manage it.

Q8 tasks and environments which exacerbate the condition — answered at Additional Comments 1 and in the foreseeable-stressors response.

Q2 capacity to follow a direction — the checklist records "Understanding instructions: Normal" and "Carrying out instructions accurately: Normal."

I do not raise this to refuse further clarification. I have set out at Part 3 what my practitioners will provide. I raise it because I have been excluded from the workplace for five weeks while information the Health Service has held since 3 July 2026 is sought again.

1.4 THREE MATTERS ON THE FACE OF THE CHECKLIST

- (a) **It records a continuation, not a return.** The first page states: "Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)."
- (b) **It records twelve months of sustained work.** At Additional Comments 3: "This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation."
- (c) **The restriction in issue asks for nothing the Health Service does not already do.** The checklist asks that complaints be "logged and redirected to management/the dedicated complaints area." The letter of 31 July states that the established process is "immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services." Those are the same arrangement. The Role Description at Attachment 2 contains no complaint-handling responsibility.

I record also that the checklist recommends, at Additional Comments 4(c), "return-to-work coordination and correspondence through the Injury Management team, Human Resources." That is the substance of what I propose at Part 7(e).

PART 2 — PROPOSED ALLOCATION

Q	SUBJECT	PROPOSED RESPONDENT
1(a)	Date of first diagnosis	Answered openly; clinical context from the psychiatrist
1(b)	Clinical basis for the causal link	Withdrawal requested — a psychiatrist question, and in issue in the appeal
1(c)	Self-report v clinical assessment; other reports held	Consented to — Dr Ma. Answer: the specialist reports he holds, not self-report
1(d)	Foreseeable risk; controls medically necessary	Psychiatrist; Dr Ma from the specialist documentation he holds
2	Following directions; performance and conduct discussions	Health Service — threshold question
3	Fitness under the existing reporting arrangements	Health Service — risk assessment, then treating psychiatrist
4	Restrictions, adjustments, duration and review date	Dr Ma (GP)
5	Meaning of "complaint handling"	Health Service — already answered, then Dr Ma
6	Requirements of the role without modification	Health Service — identify genuine occupational requirements, then GP
7	"Working memory affected under stress"	Treating psychiatrist
8	Tasks and environments that exacerbate the condition	Psychiatrist; Dr Ma from the specialist documentation he holds
9	If unable to accommodate, whether safe return is possible	Health Service — onus is the employer's

Of the nine questions, four are not medical questions. They are questions the Health Service must answer itself before any clinician can usefully be asked anything.

PART 3 — QUESTIONS FOR MY PRACTITIONERS

WHO HOLDS THE CLINICAL PICTURE

Dr Ma is my general practitioner and completed the Employee Capabilities Checklist. He can answer within the scope of general practice, and he can confirm what is on his own file.

My psychiatric condition, however, is managed by my treating psychiatrist, and Dr Ma's understanding of it derives from that specialist's correspondence to him. On the questions which turn on the nature of the condition — its clinical basis, the risk it presents, what exacerbates it, and how it affects function — Dr Ma answers from the specialist documentation he holds, and not from his own psychiatric assessment.

That is not a limitation I have created. It is how my care is arranged, and it is the reason several of these questions are better put to the psychiatrist directly.

It is also, in substance, the answer to Question 1(c). Dr Ma's view of the identified stressors does not rest solely on my self-report. It rests on the specialist reports available to him.

3.1 FOR DR MA — I will obtain answers to these

Q1(d) Whether exposure to the identified stressors presents a foreseeable risk to my health or safety on return, and what specific workplace controls or adjustments are medically necessary to manage that risk. Dr Ma will address this from the specialist documentation he holds. Because the risk in question is a psychiatric one, I will also put it to my treating psychiatrist — see Part 3.2.

Q4 The restrictions or adjustments recommended, the clinical basis in functional terms, and the anticipated duration and review date.

Q5 What specific activities are encompassed within "complaint handling" — as a matter of clinical restriction. The operational meaning has already been stated by the Health Service and is dealt with at Part 4.3.

Q8 Specific tasks, situations or environments which may exacerbate my condition. Dr Ma will address this from the specialist documentation he holds. What exacerbates a psychiatric condition is a psychiatric question, and I will put it to my treating psychiatrist as well — see Part 3.2.

Q1(c) Whether the identified stressors are based "solely on Mr Shepherd's self-report, your own clinical assessment, or other medical information/reports available to you." I do not object to this question and I consent to it being answered. Dr Ma can confirm what medical reports he holds and has relied on, and I am content for him to verify and confirm that. As set out above, the answer is that his understanding rests on the correspondence he holds from my treating psychiatrist, and not on my self-report alone. I ask only that the question be understood as directed to what Dr Ma holds and has relied upon — his own file — and not as an inquiry into material held by others, which he is not in a position to answer.

Each is necessary to identify a required adjustment or to assess the suitability of a proposed one. Each is within a general practitioner's scope.

3.2 FOR A TREATING PSYCHIATRIST — I am willing to obtain this

Q1(a) "When was Mr Shepherd first diagnosed with MDD?" I answer this openly: the diagnosis was made on 24 October 2024. Two matters should be recorded about the question as framed. First, a bare date does not identify a required adjustment or assess the suitability of a proposed one. What makes it meaningful is the clinical context — my current condition, the course of the condition since the diagnosis, and the period over which the recommended adjustments are expected to be required. That is properly a matter for my treating psychiatrist, and I ask that it be put to him in those terms. Second, the question asks my general practitioner to speak to my complete psychiatric diagnostic history — in substance, to certify that no other diagnosis exists. That is beyond a general practitioner's scope and I ask that it not be put to him in that form.

Q1(d) Whether exposure to the identified stressors presents a foreseeable risk to my health or safety on return, and what controls or adjustments are medically necessary to manage that risk. Dr Ma will answer this, and I do not ask that it be taken from him. But the risk in question is a psychiatric risk, and my treating psychiatrist is better placed to assess it and to specify the controls it requires. I will put the question to him as well, so that the Health Service has a specialist view of the adjustments it is being asked to make.

Q8 Specific tasks, situations or environments which may exacerbate my condition. As with Question 1(d), Dr Ma will answer this and I do not ask that it be taken from him. What exacerbates a psychiatric condition is a psychiatric question, and a specialist view will be of more use to the Health Service in settling the adjustments.

Q7 What "working memory is affected under stress" means in functional terms, including how it may affect capacity under normal workplace demands and the circumstances likely to cause difficulty.

This is a legitimate adjustment question, but a functional cognitive opinion is properly a psychiatrist's rather than a general practitioner's. Dr Ma can address it in general terms only.

So that it can be arranged, please confirm which questions the Health Service presses — that is, whether it presses Question 7, and identify any part of Questions 1(d), 4 or 8 on which it requires a specialist rather than a general practitioner's opinion.

I will then instruct my treating psychiatrist. The invoice for the consultation and the report will be directed to you, on the basis of the Health Service's written confirmation that it will meet my reasonable costs of preparing this report. If the Health Service contends that its undertaking does not extend to specialist input which its own questions require, I ask that it say so now, rather than after the cost has been incurred.

Consistent with option 1, I will obtain that opinion and provide it. It will be directed to my current capacity, functional restrictions and the adjustments recommended — that is, to the stated purpose of this inquiry. It is not a medico-legal opinion on the cause of my injury, which is a matter in issue in WC/2024/227 and is not necessary for that purpose.

The sequence this requires is worth stating, because it bears on the timeframe. I am not able to instruct my treating psychiatrist until the Health Service confirms which questions it presses and that it will meet the cost. Once it has, an appointment must be obtained and the opinion prepared. The same applies to Dr Ma: the consultation I cancelled cannot sensibly be rebooked until the question set is settled.

None of that can occur within seven calendar days. The only step which cannot presently begin is the one which depends on the Health Service, and I ask that it be taken promptly so that the rest can follow.

The invoice for any specialist opinion will be directed to the Injury Management Consultant on the same basis as Dr Ma's, consistent with the Health Service's confirmation that it will meet my reasonable costs.

I note that a request to my own treating practitioners rests on a different footing from an independent medical examination, which is governed by chapter 3 part 8 division 5 of the Public Sector Act 2022 and Directive 04/24, and which has not been invoked.

PART 4 — QUESTIONS FOR THE HEALTH SERVICE

These four questions ask a doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that the Health Service answer them, after which the properly medical component of each can be put to my practitioners.

4.1 QUESTION 2 — is there a performance or conduct process?

The question asks whether I can "follow a reasonable and lawful direction issued by [my] supervisor" and "participate in discussions in relation to [my] workplace performance and/or conduct."

These are not questions about capacity to perform the genuine occupational requirements of my role. They concern readiness for a management process. Either such a process is on foot concerning me — in which case I am entitled to be told of it, given particulars, and afforded procedural fairness — or there is not, in which case the questions do not serve the stated purpose of this inquiry.

→ **FOR THE HEALTH SERVICE: please confirm whether any performance or conduct process concerning me is on foot or contemplated, and if so provide particulars.**

→ **THEN, IF APPROPRIATE, FOR MY PRACTITIONER: "Does the employee have capacity to participate in ordinary workplace meetings and supervision, and are any supports required?"**

The premise of Question 2 is separately incorrect and is addressed at Part 5.

4.2 QUESTION 3 — is the workplace safe?

The question asks whether I am "medically fit to return to their substantive role under the existing reporting arrangements, including working with and reporting to their current line manager."

This asks a treating doctor to certify that a workplace arrangement is safe. Under section 19 of the Work Health and Safety Act 2011 that duty rests with the Health Service. Under the Managing the risk of psychosocial hazards at work Code of Practice 2022, a psychosocial hazard is a feature of the work, to be identified and controlled by the person conducting the business or undertaking. G03 clause 1 requires that "proper consideration must be given to the Managing the risk of psychosocial hazards at work Code of Practice 2022, to ensure the process of providing reasonable adjustments in the workplace, identifies and manages psychosocial hazards and risks."

What is sought is a workplace risk assessment, not a medical opinion. A doctor can advise on me; a doctor cannot assess the Health Service's reporting arrangements.

→ **FOR THE HEALTH SERVICE: a risk assessment of the Switchboard work environment addressing the psychosocial hazards identified. Clause 7.2.1 of the Queensland Public Health Sector Certified Agreement provides the mechanism.**

→ **THEN FOR MY PRACTITIONER: "Are any adjustments recommended in relation to supervision arrangements, and if so, what are they in functional terms?"**

4.3 QUESTION 6 — what are the genuine occupational requirements?

The question asks whether I can "fulfill the full inherent requirements of [my] Role ... without restrictions or modifications to duties."

This applies the wrong test. G03 states its purpose as assisting employees "to meet the genuine occupational requirements of their role by applying principles of reasonable adjustment," and at clause 3 provides that adjustments "should support the employee to undertake the genuine occupational requirements of their role, without risk to their own health and safety or that of others in the workplace." Section 34 of the Anti-Discrimination Act 1991 is to the same effect. Neither asks whether an employee can perform a role without adjustment.

A clinician also cannot determine which duties are genuine occupational requirements. That is the employer's determination. The Health Service has itself already made part of it: its letter states that "established process for client complaints received by Switchboard employees is for immediate escalation and management by the Health Service

Client Liaison Officer and/or Manager, Switchboard Services," and the Role Description at Attachment 2 contains no complaint-handling responsibility.

→ **FOR THE HEALTH SERVICE: identify which duties of the role it says are genuine occupational requirements, having regard to the Role Description at Attachment 2.**

→ **THEN FOR MY PRACTITIONER: "Can the employee perform the genuine occupational requirements of the role with the adjustments you have recommended?"**

4.4 QUESTION 9 — can the adjustments be accommodated?

The question asks: "If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?"

This presupposes an inability to accommodate before any accommodation has been assessed. G03 clause 2 provides that unjustifiable hardship "is tested against the whole organisation, not a division or unit within the organisation," and that "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable."

The question therefore asks my doctor to answer the final step first, on a hypothetical the Health Service has constructed and not yet tested, and on which the onus is not mine.

→ **FOR THE HEALTH SERVICE: assess the recommended adjustments against the whole organisation. If it considers any cannot be accommodated, identify which and its reasons.**

→ **THEN, IF NECESSARY, the question can be put to my practitioner properly.**

PART 5 — THE QUESTION I ASK BE WITHDRAWN, AND A CORRECTION OF FACT

5.1 WITHDRAWAL REQUESTED

Q1(b) "What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition." This is not a question a general practitioner can properly answer. The clinical basis for a psychiatric causal formulation is a matter for a psychiatrist. It is also a question I ask be withdrawn. For the purpose of identifying controls, what matters is that the stressors exacerbate my condition and what controls address them — both of which are obtained by Questions 1(d) and 8. The clinical reasoning as to causation goes to a matter in issue in WC/2024/227 and is not necessary for the stated purpose of this inquiry.

I record that section 124(1) of the Anti-Discrimination Act 1991 provides that a person "must not ask another person, either orally or in writing, to supply information on which unlawful discrimination might be based," and that under section 124(3) the defence requires the respondent to prove "that the information was reasonably required for a purpose that did not involve discrimination." Impairment is a protected attribute. I raise this not as a complaint but so that the Health Service can satisfy itself that each question it presses meets that standard.

5.2 CORRECTION OF FACT

Question 2 is premised on the statement that "the Health Service is not aware of any concerns being raised for appropriate management."

That statement is incorrect. Concerns have been raised in writing on multiple occasions, including a public interest disclosure (PID 24-ESU-1130); a written patient safety report; correspondence to Human Resources over a period exceeding two years concerning fatigue, rostering and the management of directory information; a formal complaint in February 2026; and the matters in WC/2024/227, in which the Health Service filed an objection dated 5 June 2026 under the signature of the Health Service Chief Executive.

I do not ask the Health Service to agree with any of those concerns. I ask only that my treating practitioner not be asked to answer a question which assumes they were never made. I request that the correction be provided to Dr Ma in writing before he is asked to answer any question resting on that premise.

PART 6 — DELEGATION AND CONFLICT OF INTEREST

I raised the question of who is making decisions about me, and whether any conflict of interest has been identified, in my email of [DATE]. I have not received a response. I ask again, and ask that it be answered in writing.

Both letters of 31 July 2026 are written in the first person of the Injury Management Consultant — "I am seeking further clarification", "I require further information", "I will make a decision regarding your ability to perform your role" — and are signed by Mr Hughes. I therefore understand Mr Hughes to be the delegate for the decisions those letters describe. If that is not correct, please identify the officer who is, and explain who exercises which function.

Proceeding on that understanding, the position is this.

- (a) Question 3 asks my general practitioner to advise whether I am fit to return "under the existing reporting arrangements, including working with and reporting to [my] current line manager." Those reporting arrangements sit within Corporate Services, of which Mr Hughes is the Director.
- (b) The Manager, Switchboard Services — who on 2 July 2026 required the Employee Capabilities Checklist and advised that my return to work could not be facilitated until it was provided — reports within that same directorate.
- (c) Mr Hughes is asked, at Part 4 of this document, to determine whether those arrangements are safe and whether adjustments to them can be accommodated.

The officer determining my return to duty is therefore the Director of the directorate in which the direction of 2 July 2026 was given, and the officer to whom the manager who gave it reports. He is not adjacent to the matters in issue; he sits within them.

I ask the Health Service to confirm:

- (d) the source and instrument of the delegation relied on;
- (e) whether any conflict of interest has been identified, declared and managed in relation to the decision-maker, and if so, how; and
- (f) whether the direction of 2 July 2026, and the decision of 3 July 2026 by which I was excluded from the workplace, were made with the knowledge or approval of the officer now determining my return to duty.

So far as I am aware, every decision concerning my attendance, my pay and my return to duty since 2 July 2026 has been made within Corporate Services. If that is not correct, I would be grateful to be told who else has been involved.

I ask that any decision concerning my capacity or my return to duty be made by an officer outside Corporate Services. I am not asking that anyone be found to have done anything wrong. I am asking that the decision be made by someone who is not within the arrangements it concerns. Metro South Health is a large organisation and I do not anticipate that this would present any difficulty.

I make no allegation against any individual. I ask that the position be confirmed, consistent with clause 1.2 of the Code of Conduct for the Queensland Public Service, which provides that a conflict of interest is not wrongdoing in itself, but that failing to disclose and manage one is likely to be wrongdoing. I note that the Managing the risk of psychosocial hazards at work Code of Practice 2022 identifies poor organisational justice — including a lack of procedural fairness — as a psychosocial hazard in its own right, and that G03 clause 1 requires proper consideration to be given to that Code in applying this process.

PART 7 — PROPOSAL FOR RETURN TO WORK

I have been held out of the workplace since 3 July 2026 without wages, with my accrued leave debited, and self-funding the medical costs of this process. G03 clause 1.1 contemplates temporary adjustments which allow an employee to "continue work during their recovery," and clause 1.1.1 confirms that workplace rehabilitation support is available to employees "who have an injury/illness impacting their current ability to perform their role, with no claim or entitlement to compensation (health management case)." Clause 1 provides that "Managers are responsible for ensuring work environments and work, do not unreasonably prevent employment for people."

I propose the following. Each item is within the Health Service's existing processes and, so far as I can see, requires no material expenditure and no restructuring.

- (a) RETURN TO DUTY. That I return to my rostered shifts now, on the restrictions certified in the Employee Capabilities Checklist of 3 July 2026, while the balance of this inquiry is completed. If the exclusion is maintained, that the instrument relied on be identified, that the period be treated as paid special leave, and that the leave debited since 3 July 2026 be reversed.
- (b) COMPLAINT HANDLING. That the Health Service confirm in writing that complaints received by Switchboard staff are escalated in accordance with the process it has itself described. This requires no change to anything.

- (c) ROSTER PREDICTABILITY. That my roster be published in accordance with the certified agreement and not altered without my agreement other than in an emergency.
- (d) ORDINARY PROCEDURAL FAIRNESS. That any concern about my performance or conduct be raised with me in writing, with particulars, in accordance with the applicable directive, and that I be afforded a support person.
- (e) A SEPARATE CONTACT POINT. That leave, injury management and human resources matters concerning me be handled by an officer outside Switchboard Services rather than by my line manager, who wrote to me on 2 July 2026 concerning leave types and my return to duty. This does not alter day-to-day supervision and involves no change to reporting lines.
- (f) WORKPLACE RISK ASSESSMENT. As set out at Part 4.2.
- (g) REVIEW. That the arrangements be reviewed at six weeks, or earlier if either party considers it necessary.
- (h) GRADUATED RETURN. If the Health Service prefers, I am willing to return on a graduated basis by agreement. I am not seeking to avoid any part of my role.

If any part of this proposal cannot be accommodated, I ask that the Health Service identify which part and its reasons, having regard to G03 clause 2.

PART 8 — TIME

The request allows seven calendar days from receipt, expiring 7 August 2026, and expressly contemplates a reasonable extension. I request one.

The letter was received at 11:43am on Friday 31 July, approximately two hours before a consultation with Dr Ma of which the Health Service had been notified on 30 July. Nine questions cannot be put to a practitioner on two hours' notice, and because the scope of the questions is in issue I cancelled that consultation rather than incur a cost that both the Health Service and I would have wasted. I will rebook it once the question set is settled.

I propose that the response be provided in two parts:

- (a) the questions within Dr Ma's scope — 21 days from the date the Health Service confirms the final question set; and
- (b) any question requiring specialist input — on a timeframe set once the Health Service confirms which questions it presses and that it will meet the cost.

I do not seek delay; the exclusion is costing me my income. I ask the Health Service to note that several of its questions can only be answered by a specialist, and that pressing them necessarily extends the inquiry. It cannot be reasonable to maintain an unpaid exclusion for a period created by the Health Service's own requirements.

PART 9 — DISCLOSURE AND COSTS

DISCLOSURE. I adopt option 1 and will obtain the information from my practitioners and provide it. I do not consent to option 2 and do not provide an authority for the Health Service to obtain information directly from my treating practitioners. G03 clause 3 provides that "Queensland Health acknowledges the right of the person with disability, condition, cultural considerations or neurodivergence to choose how disclosure of information occurs," and that "it may only be necessary to share information when the ability to meet genuine occupational requirements and or individual safety is at risk and reasonable adjustment is required." My election is the exercise of a right the policy expressly recognises.

USE OF THE INFORMATION. The letters state the purpose for which this information is sought — to identify and implement appropriate workplace controls or reasonable adjustments to support a return to my substantive position. Please confirm that the information obtained will be used only for that purpose, and identify any person or entity outside the Health Service to whom it is proposed to be provided. I ask this as an ordinary question about the handling of my health information, and so that I know the position before the information is obtained rather than after.

HANDLING WITHIN THE HEALTH SERVICE. The information sought is sensitive health information. G03 clause 3 recognises my right "to choose how disclosure of information occurs," and provides that "it may only be necessary to share information when the ability to meet genuine occupational requirements and or individual safety is at risk and reasonable adjustment is required."

I ask accordingly that any medical information I provide be:

- (a) held by the Injury Management Consultant on a medical-in-confidence basis;

- (b) not provided to my line manager or to Switchboard Services — a line manager requires the adjustments to be implemented, not the underlying clinical information;
- (c) accessible within the Health Service only to those who require it in order to determine what adjustments can be made; and
- (d) considered, for the purpose of any decision about my capacity or my return to duty, by an officer in relation to whom no conflict of interest arises, consistent with Part 6 above.

I raise this in the ordinary way, and not as a criticism of anyone's handling to date.

COSTS. I note and accept the confirmation that "MSH will meet your reasonable costs of preparing this report."

- (a) The consultation of 3 July 2026 which produced the Employee Capabilities Checklist was invoiced at \$150.00 and paid by me on 10 July 2026 (receipt N471065713633). I ask that it be reimbursed.
- (b) The rebooked consultation with Dr Ma, and any further consultation required to answer the Health Service's questions, will be invoiced to the Injury Management Consultant at the address nominated.
- (c) Where specialist input is required to answer the Health Service's questions, the invoice for the psychiatrist's consultation and report will likewise be directed to you, on the same basis. If the Health Service contends otherwise, I ask that it say so promptly.

RECORDS. The covering email is copied to notes@solv.com.au and refers to a transition to "Solv, a new Injury Management Case System." Please confirm what personal and health information concerning me is held in or transmitted to that system, the basis on which it is held, and the arrangements governing its use and disclosure.

PART 10 — RESPONSE REQUESTED

I ask for a written response confirming:

1. whether I am returned to my roster, and if not, the instrument relied on to maintain my exclusion and whether the period will be treated as paid special leave with my leave re-credited;
2. the delegate, the source of the delegation, how any conflict of interest has been identified and managed, and — if one is identified — that the decision be reassigned to an officer outside Corporate Services;
3. whether any performance or conduct process concerning me is on foot or contemplated;
4. which duties of my role the Health Service says are genuine occupational requirements;
5. whether the Health Service will conduct the workplace risk assessment;
6. which questions it presses and which it withdraws;
7. that the information will be used only for the stated purpose, who outside the Health Service it is proposed to be provided to, and that it will be handled within the Health Service as set out at Part 9; and
8. its response to the proposal at Part 7.

I confirm again that I am engaging with this process and want to return to work.

Cory Lea Shepherd AO3, Switchboard Services, Logan Hospital 0417 400 227 · coryshepherd1@hotmail.com